Elbow Examination — OSCE Checklist

Deakin MD2 · MM6 · Time: 14 min

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Look → Feel → Move (active, then passive) → Measure → Special tests**.

1. Introduction

☐	Wash hands; introduce self; confirm patient ID; explain and gain consent
☐	Position: usually start standing
☐	Exposure: sleeveless top at least; compare with the other side throughout
☐	Ask about pain before touching — "Where is it sore?"
☐	General inspection: well or unwell, in pain? Bedside clues (aids, splints)? Vital signs normal?

2. Examination

	Step	What to do / say	Abnormal
Look — both sides, from front, side and back			
☐	Carrying angle	Arms by sides, palms forward	Cubitus valgus / varus (old fracture)
☐	Swelling	Over the joint and olecranon	Effusion, olecranon bursitis
☐	Skin	Erythema, scars, psoriatic plaques †	
☐	Nodules	Rheumatoid nodules, gouty tophi (extensor surface)	RA, gout
☐	Wasting	Forearm muscles	
Feel			
☐	Temperature	Back of hand, compare sides	Warm = inflammation, infection
☐	Lateral epicondyle	Palpate	Tender = tennis elbow
☐	Medial epicondyle	Palpate	Tender = golfer's elbow
☐	Olecranon tip	Palpate	Bursitis, fracture
☐	Discrete swellings	Nodules, bursa	
☐	Effusion	Thumb along edge of ulna just distal to olecranon; fully extend the elbow	Palpable bulge = fluid
Move — active, then passive if limited			
☐	Flexion	"Bend your elbow" (0–150°)	
☐	Extension	"Straighten your elbow" (0°)	Loss of extension = effusion, OA
☐	Pronation	Elbows tucked in at 90°: "Turn your palms down" (0–80°)	
☐	Supination	"Turn your palms up" (0–80°)	
☐	Crepitus	Feel during passive movement	OA
☐	Measure	Limited → degrees; otherwise "full range of motion"	
☐	Ulnar nerve*	If deformity or numbness: neuro exam of the hand and arm	Ulnar nerve entrapment
Special tests			
☐	Lateral epicondylitis	Elbow 90°, forearm pronated, wrist flexed; support elbow; patient extends wrist against resistance	Pain at lateral epicondyle = tennis elbow
☐	Medial epicondylitis	Elbow 90°, forearm supinated; support elbow; patient flexes wrist against resistance	Pain at medial epicondyle = golfer's elbow

3. Closing

☐	Thank patient; help them dress; wash hands
☐	Completion: "I would examine the shoulder and wrist (joints above and below) and do a neurovascular exam of the upper limb †."
☐	Present: "Normal carrying angle with no swelling, nodules or scars. No tenderness over the epicondyles or olecranon and no effusion. Full range of flexion, extension, pronation and supination. Tests for lateral and medial epicondylitis were negative."